The examination is being carried out on the **following application documents**

Description, Pages

1-60	filed in electronic form on	19-10-2023

Sequence listings, SEQ ID NO

1-50	filed with entry into the regional phase before the EPO

Claims, Numbers

1-14	filed with entry into the regional phase before the EPO

Drawings, Sheets

1/7-7/7	filed with entry into the regional phase before the EPO

1 The amendments filed on 19-10-2023 comply with Art. 123(2) EPC

2 Reference is made to the following document; the numbering will be adhered to in the rest of the procedure.

D1: Honda et al, 2015. Sci Rep, 5, 15921, 1-15 [XP055484189]

D2: Sato et al, 2020. Cancers, 12, 2625, 1-16 [XP055970078]

D3: Honda et al, 2016. Biomark Med, 10(11), 1197-1207 [XP055484186]

D4: WO2015050107A1

3 The subject-matter of claims 1-2, 7-11, 13 is not novel (Art 54(1) and (2) EPC).

3.1 D1 (pg 2, para 6 ; pg 8, para 3-4 ; Fig 1) discloses the use measurement of ApoA2 isoforms (namely ApoA2-ATQ and ApoA2-AT) via ELISA (i.e use of antibodies specific for the isoforms) for the detection of pancreatic cancer (e.g. malignant IPMN) and distinction of malignant from benign lesions. Malignant lesions are associated with a decrease in ApoA2-AT and an increase in ApoA2-ATQ. This document anticipates the subject-matter of claims 1-2, 7-10.

3.2 D2 (pg 3, para 2.3 ; pg 10, para 1-2 ; pg 13, para 4.7) discloses the measurement of plasma apoA2-ATQ and apoA2-AT to diagnose malignant pancreatic cancer in subjects having i.a. IPMN. This document anticipates the subject-matter of claims 1-2, 7-10.

3.3 D3 (abstract ; pg 1199, col 2, para bridging pg 1200 : pg 1200. col 2, para bridging pg 1201 ; Fig 2-3) reviews the link between apolipoprotein A2 isoforms (including ApoA2-ATQ and apoA2-AT) and pancreatic cancer as well a their use for risk stratification in i.a. patient with IPMN and MCN. The document also discloses ELISA detection (i.e. use of antibodies) of these biomarkers in blood samples. This document anticipates the subject-matter of claims 1-2, 7-10.

3.4 D4 (example 1-5 ; Fig 1-3) discloses antibodies against ApoA2-ATQ (e.g. /F2, 6G2), ApoA2-AT and ApoA2, as well as their use for the detection of pancreatic tumor in blood samples from patients. This document anticipates the subject-matter of claims 9-11, 13.

4 Dependent claims 3-6, 12 and independent claim 14 do not appear to contain any additional features which, in combination with the features of any claim to which they refer, meet the with respect to inventive step, as the relevant subject matter falls within the knowledge and ability of the skilled person, starting from e.g. D1. Indeed, once the link between the biomarkers ApoA2-ATQ and ApoA2-AT and pancreatic cystic tumors is known, it lies within the knowledge and abilities of the skilled person to chose appropriate patient groups for comparison, thereby arriving at subject-matter falling within the claimed scope without exercising inventive skill. No technical prejudice had to be overcome and no surprising advantages are shown. In addition, the different biomarkers being known (e.g. D1-D4 cited above), as well as their sequence, the skilled person is in a position, based upon his common general knowledge to prepare antibodies for their detection. Here again, no technical prejudice had to be overcome and no surprising advantages are shown.

5 Art 84 EPC

5.1 The C-terminal regions of ApoA2, ApoAT and Apo-ATQ only differ in maximum 3 C terminal amino acids. There are thus many overlapping epitopes. As a result, the following considerations apply: Mere definition binding to SEQ ID 31 is not sufficient to ensure detects specifically ApoA2-ATQ. Indeed, the C-terminal region of ApoA2 ATQ de facto also comprises the "shorter" versions ApoA2 and ApoA2-AT. In fact, ApoA2-ATQ and ApoA2-AT only differ in 1 amino acid and thus have many overlapping epitopes. As a result, mere binding to SEQ ID 31 is not sufficient to make sure it in fact detects ApoA2-ATQ, because other not unique epitopes for ApoA2-ATQ are present therein.

5.2 The same observations apply with reference to SEQ ID 30 and detection of ApoA2-AT.

5.3 The subject-matter of claim 8 would appear to be contradictory with the subject-matter of claim 1, which already requires that ApoA2 ATQ or ApoA2 AT are determined.

5.4 The subject-matter of claim 13 is not clear. In particular, it is not clear whether the anti-ApoA2 non-terminus monoclonal antibody does or does not bind to the APOA2-ATQ protein consisting of the amino acid sequence represented by SEQ ID NO: 2.

6 The applicant is kindly requested to file new claims which address the above observations.